

Office Visit 11/21/2024
Pav CC Head, Neck & Respiratory

Provider: Miller, Jordan D, DO (Cardiothoracic Surgery)
Primary diagnosis: Lung nodule
Reason for Visit: Follow-up

Progress Notes

Miller, Jordan D, DO (Physician) • Thoracic Surgery



**UK College of Medicine
Department of Surgery
Section of Thoracic Surgery
Outpatient Clinic Note**

Diagnosis: Pulmonary nodule

Procedure: November 1, 2024 right VATS upper lobe wedge resection with mediastinal lymph node dissection

Pathology: Pathologic T1b N0 M0 neuroendocrine tumor not otherwise specified at this time

Interval History: [REDACTED] is a 71 y.o. male with a history of a right upper lobe pulmonary nodule who recently underwent a right upper lobe wedge resection for diagnosis and therapeutic intent. He recovered quite well and was discharged home. Since discharge she is overall doing well and offers no complaints. His pain is relieved with oral pain medication

ROS:

General: no fevers or chills, no heat or cold intolerance, no subjective weight loss

HEENT: no changes in vision, no sore throat, no changes in hearing, no tinnitus, no nasal drainage

CV: no chest pain, no palpitations, no lightheadedness, no PND, no orthopnea, no LE swelling, no claudication

Pulm: no shortness of breath, no cough, no hemoptysis

GI: no nausea, no vomiting, no abdominal pain, no constipation, no diarrhea, no melena, no hematochezia, no dysphagia, no heartburn

Skin: no rash

Neuro: no numbness, no tingling, no headache, no difficulties with speech, no gait disturbance

Heme: no easy bruising, no bleeding from the gums

Endo: No polyuria or polydypsia

Psych: no depression or anxiety

Physical exam:

Visit Vitals

BP	(!) 173/93 (BP Location: Right arm)
Pulse	75
Temp	36.9 °C (98.4 °F) (Oral)
Wt	72.2 kg (159 lb 2.8 oz)

Comment: PT states he feels fine, no symptoms. He stated his BP usually runs high when he arrives here.

SpO2 98%
BMI 24.20 kg/m²

General: alert and oriented, appropriate
Lungs: CTA B, no wheezes or rhonchi
Heart: RRR, no murmurs
Abdomen: soft NT/ND, normal bowel sounds
Lymph nodes: no palpable supraclavicular or cervical adenopathy
Extremities: no peripheral edema
Skin: no rash, no cyanosis and warm to touch
Psychiatric: oriented to person/place/time and normal mood/affect
Incisions: healing well

Imaging:

I personally reviewed the chest x-ray which shows expected changes without significant effusion or pneumothorax

Assessment and Plan:

██████████ is a 71 y.o. male who presents to the thoracic surgical clinic today for follow-up after undergoing a right upper lobe wedge resection. Pathology demonstrates a neuroendocrine tumor that is not otherwise specified at this time after discussing his case with pulmonology we will plan for CARIS testing and referral to tumor board for further treatment plans versus continued observation. At this point in time we will plan for the patient to return to clinic in 4 months with a CT of the thorax for continued oncologic surveillance

Images and pathology reviewed and discussed with the patient at today's visit and all questions answered. Patient will return to clinic in 4 months with chest CT without contrast. The patient understands this plan and will call our office for any additional questions or concerns.

Jordan D Miller, DO
11/22/24 2:12 PM

Instructions

AVS - Outpatient (Automatic SnapShot taken 11/22/2024)

Additional Documentation

Vitals: BP **173/93 !** (Abnormal) (BP Location: Right arm) Pulse 75 Temp 36.9 °C (98.4 °F) (Oral) Resp 20
Wt 72.2 kg (159 lb 2.8 oz) SpO2 98% BMI 24.20 kg/m² BSA 1.86 m² Pain Sc 0-No pain
Flowsheets: Patient-Reported Data, Vital Signs, Vitals Reassessment
SmartForms: AMB SF STEADI FALL RISK

Communications

✉ Letter sent to Sarah L Florence, APRN

📎 Provider Notes
Sent 11/22/2024

Media

From this encounter

Surprise Billing Notification - Electronic signature on 11/20/2024 10:57 AM - E-signed

Welcome Accident Related Questionnaire

Question

11/20/2024 10:49 AM EST

- Filed by Patient

Is this visit related to a workers' compensation, auto, or Black Lung?

No

Mc Patient-Generated Subjective Global Assessment (Pg-Sga)

Question

11/20/2024 10:54 AM EST

- Filed by Patient

Weight: In summary of my current and recent weight:

Please select the type of measurement system you will be using for the next four questions regarding height and weight. lbs/ft

I currently weigh approximately: 155

My current height is approximately: 5' 7"

One month ago I weighed approximately: 155

Six months ago I weighed approximately: 155

During the past two weeks my weight has: not changed

Food intake

As compared to my normal intake, I would rate my food intake during the past month as: unchanged

I am now taking

Symptoms: I have had the following problems that have kept me from eating enough during the past two weeks:

No problems eating Yes

No appetite, just did not feel like eating No

Nausea No

Constipation No

Mouth sores No

Things taste funny or have no taste No

Problems swallowing No

Pain Yes

Where is your pain located? right side

Vomiting No

Diarrhea No

Dry mouth No

Smells bother me No

Feel full quickly No

Fatigue No

Other (Examples: depression, money, or dental problems) No

Activities and Function

Over the past month, I would generally rate my activity as: not my normal self, but able to be up and about with fairly normal activities

Education

No education to display

Orders Placed



CT Chest wo IV Contrast
Clinic Appointment Request Follow up; MILLER, JORDAN D

Medication Changes

As of 11/22/2024 9:59 AM

None

Medication List at End of Visit

As of 11/22/2024 9:59 AM

	Refills	Start Date	End Date
oxyCODONE (Roxicodone) 5 MG immediate release tablet	0	11/3/2024	—
Take 1 tablet (5 mg) by mouth every 6 (six) hours if needed (mild - moderate pain) for up to 20 doses. - Oral			
Patient not taking: Reported on 11/21/2024			

Visit Diagnoses

Primary: Lung nodule R91.1